

PROTOCOL REVIEW THRESHOLD: 2.5 mg

Cohort 48

Internal Research Literature Matrix

<u>Source</u>	<u>design</u>	<u>cohort</u>	<u>threshold</u>	<u>signal</u>	<u>notes</u>
Alpha-A	Randomized parallel double-blind placebo-controlled	Cohort 48 (n = 96) balanced	2.5 mg ☒ meets threshold	↑ Primary response improvement vs placebo (p < 0.05)	- Clean randomization - No major safety flags - Consistent across key endpoints
Alpha-B	Open-label dose escalation (adaptive)	Cohort 48 (n = 72) enriched population	2.5 mg ☒ meets threshold	↑ Dose-response trend observed up to 2.5 mg	- Tolerability acceptable - Exposure increases ~linear - Watch higher doses
Alpha-C	Crossover within-subject (2-period)	Cohort 48 (n = 40) completers	2.5 mg ☒ meets threshold	→ No significant difference vs control	- Higher variability - May be under-powered - Exploratory only
Alpha-D	Observational real-world (retrospective)	Cohort 48 (n = 128) all-comers	2.5 mg ☐ does not meet threshold	↓ Weaker signal after adjustment for confounders	- Confounding likely - Data completeness variable Strong signal confirmed

Summary

- ☒ Alpha-A supports 2.5 mg protocol
- ☒ Alpha-B supports dose selection at 2.5 mg
- ☒ Alpha-C inconclusive
- ☐ Alpha-D does not support 2.5 mg threshold

Decision Guide

If ≥ 2 of 4 sources meet threshold

→ 2.5 mg acceptable

If < 2 of 4 sources meet threshold

→ re-evaluate protocol

Notes

- Threshold defined as pre-specified minimum efficacy with acceptable safety.
- Cohort 48 is the primary analysis set for this review.
- All signals are hypothesis-generating.

Re-check data lock status before final sign-off.